

NURSING VALUE ASSESSMENT

Deaconess

32 beds · 36 nurse FTEs · Inpatient Nursing

PREPARED BY

Christina Searle · 4/28/2026 · April 28, 2026



Where Nursing Value Actually Lives

Ambient documentation creates value across four distinct buckets. For nursing, three of them carry real dollars — and one of them, honestly, doesn't apply. We separate them because the strategic implications of each are different.

Nurses don't bill. So where does the value live?

Nursing documentation creates value through quality (preventable harm and bundle compliance), workforce (retention, agency, and overtime), and capacity (more time at the bedside). Revenue — the fourth bucket — is where nursing intentionally doesn't play.

CAPACITY

Tracked

Reclaimed documentation time goes back to bedside care. We track the ratio of bedside-to-charting time post-deployment as a leading indicator that the model is working.

WORKFORCE

\$50K

Documentation burden is among the top drivers of nurse turnover. The hypothesis we model: reducing end-of-shift charting moves retention, the agency spend that follows every vacancy, and the overtime budget — three distinct labor lines that all share one root cause.

QUALITY

\$55K (potential)

Preventable harm events — HAPIs, falls, CAUTI, CLABSI — and sepsis bundle compliance are sensitive to documentation timing. The hypothesis we model: real-time flowsheet capture surfaces the visibility for earlier intervention, with each unit's actual outcome shaped by clinical practice.

REVENUE

Not Applicable

Nursing documentation doesn't generate billing revenue. The value lives in labor economics and care quality — which is where we've modeled it.

WHAT YOUR CHOICES REVEAL

Your model combines overtime reduction with retention savings. OT reduction is grounded in your current overtime spend; retention reflects the burnout-related turnover that documentation burden accelerates. Together they form a defensible staffing ROI.

CAPACITY

Bedside time is what nurses say they don't have enough of.

Capacity for nursing isn't a billable line — it's a leading indicator. When nurses finish charting before the end of their shift, more of the next hour goes to direct patient care instead of catch-up documentation. We track this post-deployment to confirm the model is producing the behavior change it predicts.

HOURS RETURNED	PATIENT DAYS	MIN / SHIFT
1,685	10,512	30
annually	per year	saved per shift

BEDSIDE / DIRECT CARE TIME

TRACKED

Reclaimed documentation time spent in direct patient care. Track the ratio post-deployment as the leading indicator that ambient documentation is producing the behavior change.

Documentation burden is the reason nurses leave. And the reason they stay late.

Three distinct budget lines — turnover cost, agency premium, and overtime — all driven by how long it takes to finish charting at the end of a shift. Reducing the documentation burden moves all three.

RN RETENTION

\$22K

Documentation burden is a leading driver of nurse burnout. Burnout drives turnover. Reducing burden helps retain experienced nurses who would otherwise leave.

36 nurses × 18% turnover × 40% burnout-related × 15% impact × \$56,300/replacement
Source: NSI Nursing Solutions 2024 turnover benchmark.

OVERTIME REDUCTION

\$28K

When nurses spend less time documenting at end of shift, OT hours decrease. Direct, measurable line item in the payroll budget.

36 nurses × 0.5 OT hrs/week × 40% reduction × 52 weeks × \$75/hr

WORKFORCE SUBTOTAL \$50K

QUALITY

Real-time documentation is the visibility layer that makes early intervention possible.

Preventable harm events don't happen because nurses don't care — they happen when risk signals are missed or delayed. The hypothesis we model: real-time flowsheet capture surfaces those signals while there's still time to act, with the actual outcome determined by clinical practice on each unit.

The values below are potential — they require clinical practice change alongside documentation improvement. Documentation creates the visibility; the care team creates the outcome. Validate baseline rates with your infection-control and quality teams before presenting.

HARM EVENTS

HAPI RISK REDUCTION

\$41K (potential)

At 2.4/1,000 patient days across 10,512 patient days, your program sees approximately 25.2 HAPIs per year. The hypothesis we model: when skin assessments are captured at the point of care rather than reconstructed at shift end, the visibility for earlier intervention improves. At a 6.5% documentation-attributable prevention rate and \$25,000/event, that translates to \$41K — a potential to validate against your wound-care team's baseline.

Source: Dowding et al., JAMIA 2012.

FALL RISK VISIBILITY

\$14K (potential)

At 2.03/1,000 patient days, your program sees approximately 21.3 falls per year. The hypothesis we model: when Morse Fall Scale assessments are completed in real time rather than deferred to end of shift, risk escalations become visible to the care team when they matter. At a 10% documentation-attributable prevention rate and \$6,500/event, that translates to \$14K — a potential to validate against your falls-prevention committee's data.

Source: AHRQ inpatient fall cost benchmarks.

BUNDLE COMPLIANCE

No bundle-compliance drivers selected for this assessment.

TRACKED METRICS

HCAHPS / PATIENT EXPERIENCE

Qualitative

Patient experience scores are sensitive to nursing presence and communication — both of which improve when nurses spend less time at the workstation. HCAHPS performance affects Value-Based Purchasing scores and thus Medicare reimbursement, but the causal chain is indirect and organization-specific to size precisely.

Nursing documentation doesn't generate billing revenue.

That's by design — and it's worth saying directly.

Every other care setting in this model has a revenue optimization story — wRVU lift, DRG accuracy, denial prevention. Nursing doesn't. Nurses don't bill. Their documentation drives care quality and labor economics, not the revenue cycle.

We've modeled the value where it actually lives: in the QUALITY bucket (preventable harm events, sepsis bundle compliance) and the WORKFORCE bucket (retention, agency cost avoidance, overtime reduction). Presenting a nursing revenue story that doesn't exist would undermine the credibility of the model that does.

WHAT THIS MEANS FOR YOUR PRESENTATION

When a CFO asks where the nursing revenue story is, the answer is: there isn't one — and that's the point. The ROI case for nursing ambient AI is a labor economics and care quality story. It stands on its own. You don't need to manufacture a billing narrative to make it compelling.

THE INDIRECT CONNECTION

Complete nursing documentation does create downstream organizational value: HCAHPS performance affects Value-Based Purchasing scores. HAC scores affect Medicare payment rates. Sepsis SEP-1 compliance affects public reporting and payer relationships. These are real — they're just not direct billing revenue, and we've modeled them as quality outcomes rather than revenue line items, which is the more defensible framing.

THE INVESTMENT CASE

Infrastructure, Not Expense.

The investment is \$52K annually – \$136/bed/month at 32 staffed beds. A one-time implementation fee of \$50K applies separately. Years 2–3 assume 10% growth as adoption matures and documentation habits stabilize across the unit.

PERIOD	VALUE	INVESTMENT	NET VALUE	CUMULATIVE
Year 1	\$105K	\$52K	\$53K	\$53K
Year 2	\$115K	\$52K	\$63K	\$116K
Year 3	\$127K	\$52K	\$75K	\$190K

By Year 3, the program is generating 1.2× cumulative net for every \$1 invested – while your nursing staff spends more time at the bedside and less time charting after their shift.

AT SCALE

CURRENT MODEL

\$105K/yr

32 beds · 60% adoption

Per bed: \$1,644/yr

AT FULL SCALE

\$419K/yr

128 beds · 80% adoption

Per bed: \$1,644/yr

Illustrative scaling, not a hard projection.

KEY METRICS TO TRACK

- 1. Documentation time per shift (target: –40%)
- 2. Nurse satisfaction / burnout score (target: +15 pts)
- 3. Overtime hours per FTE per week (target: –25%)

YOUR ASSESSMENT SUMMARY

32 staffed beds · 36 nurse FTEs.

\$53K projected net value

\$1,644/bed per year

VALUE SUMMARY

QUALITY	\$55K (potential)
HAPI Prevention	\$41K
Falls Prevention	\$14K
CAUTI Prevention	—
CLABSI Prevention	—
Sepsis Bundle Compliance	—
HCAHPS / Patient Experience	Qualitative
Medication Error Reduction	—
WORKFORCE	\$50K
RN Retention	\$22K
Agency & Travel Nurse Reduction	—
Overtime Reduction	\$28K
CAPACITY	Tracked
Bedside / Direct Care Time	Tracked
REVENUE	Not Applicable
Nursing documentation does not generate billing revenue.	

NET ANNUAL VALUE **\$53K**

METHODOLOGY

- HAPI: 2.4/1,000 patient days. 6.5% documentation-attributable prevention rate. \$25,000/event. Source: Dowding et al., JAMIA 2012.
- Falls: 2.03/1,000 patient days. 10% documentation-attributable prevention rate. \$6,500/event. Source: AHRQ inpatient fall cost benchmarks.
- HCAHPS: Tracked qualitatively. Patient-experience scores affect Value-Based Purchasing but the causal chain to documentation is indirect; no monetary impact is modeled.

- Retention: 18% annual turnover, 40% burnout-related. 15% impact scenario. \$56,300 replacement cost. Source: NSI Nursing Solutions 2024.
- OT Reduction: 0.5 OT hrs/nurse/week at 40% reduction. \$75/hour.
- Revenue: Not applicable. Nursing documentation does not generate billing revenue.

This assessment is for planning purposes. Hard value projections are based on user-provided staffing inputs. Potential value uses published clinical rates with documentation-attributable prevention rates that reflect the indirect causal chain. Validate with your organization's data post-implementation.